



PEDIATRIC SHOCK

Effective: January 1, 2026

Replaces: January 1, 2025

1. Patient Care Goals

- 1.1. Initiate early fluid resuscitation and vasopressors to maintain/restore adequate perfusion to vital organs
- 1.2. Differentiate between possible underlying causes of shock to promptly initiate additional therapy

2. BLS Treatment

- 2.1. Routine Medical Care – Pediatric (700-S05)
 - 2.1.1. **Oxygen High Flow** – and assist with ventilations as appropriate
- 2.2. Place patient in shock position
- 2.3. Control any obvious bleeding as appropriate
 - 2.3.1. Consider the use of a tourniquet if bleeding is not controlled (700-M17)
- 2.4. Maintain patient's body temperature
- 2.5. If Anaphylaxis is suspected see treatment protocol (700-P12)
- 2.6. If Trauma is suspected see treatment protocol (700-P16)

3. ALS Treatment

- 3.1. **Vascular Access (IV) or Vascular Access (IO)**, per procedure (700-M13)
- 3.2. Consider fluid bolus:
 - 3.2.1. Neonatal: **10 ml/kg Normal Saline IV/IO**
 - 3.2.2. Infant and Child: **20 ml/kg Normal Saline IV/IO**, max of 60 ml/kg
 - 3.2.3. May repeat fluid bolus if lung sounds remain clear to maintain normal systolic blood pressure (SBP)
- 3.3. If unable to maintain normal SBP and/or heart rate per (700-S05) after fluid bolus, **Push-dose Epinephrine (0.01 mg/ml)**,
 - 3.3.1. Mix **9 ml Normal Saline** with **1 ml Epinephrine 1:10,000 IV/IO** formulation in a 10 ml syringe
 - 3.3.1.1. Label syringe "**Epinephrine 0.01 mg/ml**"
 - 3.3.1.2. Administer **0.1 ml/kg**, every 1-5 minutes, max dose 1 ml
 - 3.3.1.3.

Weight in kg	Dose in ml	Dose in mg (Document This)
3	0.3	0.003
4	0.4	0.004
5	0.5	0.005
6	0.6	0.006
7	0.7	0.007
8	0.8	0.008
9	0.9	0.009
10+	1	0.01

4. Pertinent Assessment Findings

- 4.1. Decreased perfusion manifested by altered mental status, or abnormalities in capillary refill or pulses, decreased urine output



- 4.1.1. Cardiogenic, hypovolemic, obstructive shock: capillary refill greater than 2 seconds, diminished peripheral pulses, mottled cool extremities
- 4.1.2. Distributive shock: flash capillary refill, bounding peripheral pulses

5. Key Documentation Elements

- 5.1. Medications administered
- 5.2. Vital signs according to Routine Medical Care – Pediatric (700-S05)
- 5.3. Neurologic status assessment
- 5.4. Amount of fluids given